

DISCHARGE SUMMARY

Patient: Arjun Mehta MRN: SYN-DC-1001 DOB: 1978-03-14
Admission Date: 2026-04-02 Discharge Date: 2026-04-07
Attending Physician: Dr. S. Kapoor, Internal Medicine

PRIMARY DIAGNOSIS

Acute exacerbation of chronic obstructive pulmonary disease (COPD), GOLD stage III.

SECONDARY DIAGNOSES

1. Type 2 diabetes mellitus, well-controlled.
2. Essential hypertension.
3. Mild normocytic anemia, likely chronic disease related.

HOSPITAL COURSE

Patient presented to the emergency department with a three-day history of worsening dyspnea, productive cough with yellow sputum, and wheeze. On admission, oxygen saturation was 88% on room air, improving to 94% on 2L nasal cannula. Chest X-ray showed hyperinflated lung fields without consolidation. Patient was started on nebulized bronchodilators (albuterol/ipratropium), intravenous methylprednisolone 40mg daily, and empiric azithromycin for possible bacterial component. Symptoms improved steadily over 5 days. Patient was transitioned to oral prednisone taper and discharged home in stable condition, ambulating independently on room air.

DISCHARGE MEDICATIONS

1. Prednisone 40mg PO daily, taper by 10mg every 3 days.
2. Tiotropium inhaler, one puff daily.
3. Albuterol inhaler, 2 puffs every 4-6 hours as needed.
4. Metformin 500mg PO twice daily (home medication, continued).
5. Lisinopril 10mg PO daily (home medication, continued).

FOLLOW-UP INSTRUCTIONS

Follow up with pulmonology in 2 weeks for spirometry reassessment. Follow up with primary care physician in 1 week. Patient advised on smoking cessation resources (patient is a former smoker, quit 2019). Return to ED immediately if oxygen saturation drops below 90% at home or if symptoms worsen significantly.